

State Policy and Political Mis(Education)

In January 2015, Mike Pence, then the governor of Indiana, announced that his state would extend Medicaid to everyone living below 138 percent of the federal poverty line. Following the prescriptions of the ACA, the Healthy Indiana Plan pledged to insure an additional 350,000 low-income people, but there were a few caveats. As a Republican eager to distance himself from “Obamacare,” Pence proudly touted the modifications that Indiana would make to the program. Beneficiaries would be required to pay monthly premiums amounting to 2 percent of their household income. Those unable or unwilling to pay would only be eligible for downgraded coverage (which excluded dental and vision and required co-payments up to \$75). Moreover, benefits could be suspended for up to six months as a penalty for nonpayment. In Governor Pence’s sanguine view, this reform would prompt people to “take greater ownership of their health care.” Looking beyond Indiana, critics decried a more worrisome outcome: a “many-headed Medicaid” marked by different standards in different states and spawning increasing inequality across the nation (Sanger-Katz 2015). In Chapter 3, I sketched a portrait of precisely such a landscape (historically and contemporarily). In this chapter, I investigate the democratic consequences of Medicaid policy fragmentation by examining how state policy disparities bear upon beneficiaries’ political behavior.

MEDICAID, FEDERALISM, AND POLICY FEEDBACK

For more than two decades, scholars have marshaled robust evidence that policies affect citizens’ political behavior (Bruch, Ferree, and Soss 2010; Campbell 2003, 2012; Mettler 2005; Pierson 1993; Soss 1999,

2000; Weaver and Lerman 2010). Such research has shown that the structure and design of government programs can influence civic and political participation by channeling resources, generating interests, and shaping interpretive schemas (Béland 2010; Campbell 2012; Mettler and Soss 2004; Pierson 1993; Schneider and Ingram 1993). Though impressive in range and depth, most studies of policy feedback have paid little attention to subnational differences in policies.¹

More generally, theories of policy feedback provide useful but indeterminate guidance about the micro-political effects of Medicaid. Much of the literature implies that as a means-tested program, Medicaid is likely to depress the political participation of those who enroll (Campbell 2012; Mettler 2002; Mettler and Stonecash 2008; Soss 1999). However, Medicaid has fared well in the court of public opinion and years of research confirm that it is less stigmatized than other means-tested programs (Campbell 2015; Cook and Barrett 1992; Grogan and Patashnik 2003; Howard 2006; Stuber and Kronebusch 2004). Moreover, burgeoning scholarship examining the short-term aggregate participatory effects of recent Medicaid expansions has found an increased likelihood of districtwide and countywide voting in the wake of coverage expansions (Clinton and Sances 2017; Haselswerdt 2017). However, though the immediate positive effects of *coverage expansion* may indicate an electorate that is responsive to major policy changes, such effects do not speak to the more enduring consequences of the *experiences* that emerge from actually using Medicaid. In short, coverage effects are quite distinct from the *experiential* effects of policy utilization. To date, political scientists have only probed the former.² Altogether, the research record does not offer a clear and comprehensive view of the effect of Medicaid on individual-level political participation.

Perhaps paradoxically, I seek to further complicate this literature by suggesting that uncovering Medicaid's broad political effects is a less pressing task than understanding how those effects are geographically distributed. There is no uniform Medicaid program. Each state runs its own program and (as I detailed in Chapter 3) they vary substantially. We should therefore approach overly expansive assessments of Medicaid's policy feedback processes with some trepidation.

MEDICAID AND POLITICAL PARTICIPATION: THEORIZING BEYOND THE GENERAL

I began this research with a very general question: how do experiences with Medicaid affect political behavior? I then embarked on a series of

in-depth interviews with beneficiaries. At that point, I knew enough to deduce that since Medicaid was a federal-state program, I should interview beneficiaries in a range of places. This was a commonsense intuition about case selection, but I originally put little theoretical emphasis on geography. Instead, I was searching for a general theory of the Medicaid-to-politics link. Then I began talking to people. One of the first beneficiaries I spoke to was John, whom I introduced in Chapter 1. John perceived stigma in relying on Medicaid and sometimes had negative bureaucratic interactions. Though he had a chronic debilitating illness, John was a young (in his thirties) white man who at times appeared healthy, and he believed that this aroused suspicions among bureaucrats:

I've had a few instances where they [caseworkers], I know I was just looked at differently because I am a male and I think it was more or less the women, you know; I hate to say that, but dealing with what I've dealt with, like when an older woman if I'm assigned to her [as a caseworker] she just looks me up and down and I kind of, I kind of look better [health wise] than I am you know.

This subtle invocation of stigma was not surprising given that Medicaid is a means-tested public program (Mettler 2002; Mettler and Stonecash 2008). In addition, John mentioned administrative burdens and negative evaluations of caseworkers (Burden et al. 2012; Moynihan, Herd, and Harvey 2014):

As far as like the caseworkers and the paperwork ... all the caseworkers quit all the time and they're sick of dealing with everybody, and everybody hates each other; it's been just like a circus, like a zoo ... the paperwork is fill this out, come here, and then you know, home inspections, they come to my house and I have to have my doctors fill this out, and it's yeah, it's been getting pretty rough.

Near the end of the interview, John noted that he did not believe Medicaid beneficiaries have much power: "No, I don't think that we are going to change anything as patients." He also admitted that he made little investment in electoral politics:

I have listened to a few speeches or whatever on TV and I honestly can't tell you anything about what they support or if they are Democrat or Republican ... I am not involved and I know I probably should be.

Coming early in the research process, my conversation with John signaled some of what I surmised dampened Medicaid beneficiaries' political participation: stigma and negative interactions with state government. However, John's narrative also contained elements that I had not anticipated: a declaration that he was "married" to Michigan medically and an account of an attempt to move to Arizona that was foiled by the

impossibility of getting equivalent Medicaid benefits there. I was especially struck by how profoundly this cross-state limitation seemed to drive John's sense of his life prospects:

I guess that's pretty much the boat I'm in now. I'm just, I'm not where I want to be, but you know, I guess beggars can't be choosers, and I try to tell myself, you know, I'll also live on Mars the second I have to and you're just going to do what you got to do and it's just not very fun, but okay unless something changes you know ... if they did like a federal insurance and everybody had their insurance card regardless of who we are, where you are, I think it would solve a lot of problems for people, but I don't think that's going to happen ... being a man, you know and being older and being like, just knowing that you're never going to better yourself ... in that sense it's been hard ... seeing other family and friends and they're able to move on ... you know it is tough. You know, it's hard to, I mean I've accepted it a long time ago because it is what it is and I've always known that you know, that's just how it is.

After talking to John, I had a hunch that Medicaid's policy feedback process was in some ways akin to that of other means-tested programs, but I was not clear about the role of geography. I kept this in the back of my mind, but did not yet perceive it as central.

Going forward, my qualitative research took me to Florida, Georgia, Idaho, New York, and more. Across all of these places, Medicaid beneficiaries continued to describe their experiences in terms of stigma and negative bureaucratic interactions. In Table 4.1, I offer just a few illustrations. Enumerating everything relevant that came up in the interviews would take too many pages, but these examples are reflective of the kinds of statements that generally emerged in my conversations with beneficiaries.

Given the proliferation of such attitudes, I developed the general hypothesis (which I provide quantitative evidence for later in this chapter) that the individual-level relationship between Medicaid enrollment and political participation would be negative (H1). Comparing beneficiaries to similarly situated non-beneficiaries, I expected that the former would be less likely to participate in politics because of their stigmatized identities as policy beneficiaries and their negative interactions with government bureaucrats.

By the conclusion of my interviews, however, I was not as interested in this general hypothesis as I was intrigued by the stark surfacing of something that I did not expect: vast geographic heterogeneity in the policy experiences Medicaid beneficiaries recounted. It was the observation of such discrepancies that led me to contemplate the relevance of

TABLE 4.1 *Qualitative Descriptions of Stigma and Bureaucratic Interactions***Stigma**

Terrie: “It’s demeaning, you know; the process is extra-extensive, long.”

Louisa: “You feel embarrassed.”

Ahmad: “They treat us like we are stupid animals; we don’t know anything ...

This is what I feel ... I feel like shit. I feel like I’m nothing, because when you are in Medicaid, they do whatever. You have to be on their rules.”

Daphne: “I think it does come with a negative stigma. Like they talk about it at school in economics and stuff, just in my business classes, and just hearing my classmates talk about it.”

Administrative Burden/Negative Bureaucratic Interactions

Roxy: “It’s a lot of paperwork. You have to have a physical, you have to have proof of your pregnancy ... You have to have proof of your income. You have to have your bills, how much you spend per your paycheck.”

Dani: “It’s a lot of work. They make it a lot of work. They want to see everything. They want to make sure that you really qualify for what they’re offering. So it’s a lot of work.”

Kay: “I distinctly remember taking the binder ... they gave us a binder and said, ‘Start working on this.’ And it was the Medicaid application, and it was so complex and there were so many pages, and I was like, it can’t be this difficult.”

Nessa: “And they[’re] so rude and nasty; that’s why I try not to come up to them ‘cause they are rude and they’re nasty ... I just hate it and so I try not to go up there.”

Katrina: “The people [who] are working in these offices ... there’s sometimes a respect issue, and there’s sometimes just a – I don’t know. When I say it felt like they were stupid, part of that is coming from a place of real frustration, and part of it is just the way they talked, which was irritating to me because it felt like the future of my health is in your hands right now, so don’t talk to me like this.”

federalism and to theorize beyond a general Medicaid-to-politics link. Instead, I redirected my focus to the reasons why Medicaid beneficiaries’ interpretations of government and politics seemed especially dependent on where they lived. In making this theoretical shift, I heeded the encouragement of Mettler and Soss (2004: 64), who suggested that scholars could enrich the study of policy feedback by recognizing that “policy-based resources may have very different political effects ... clients in a single public program, for example, may draw different lessons from their encounters.” In what follows, I demonstrate that state context is one avenue through which divergent lessons can be conveyed to beneficiaries in the same program.

A POLITICAL LEARNING APPROACH

I begin with qualitative underpinnings for my arguments. The case study data used in this book are based on sixty-one in-depth interviews with forty-five Medicaid beneficiaries and sixteen key stakeholders.³ To someone who is unfamiliar with qualitative methods, a quick reaction to these numbers may be to think, “What can you really learn from talking to 61 people?” The answer is that you can learn a lot. The value of these interviews was not in the number of cases. The goal of the qualitative component of this work was depth, not breadth. Neither achieving representativeness nor obtaining a massive “sample” were my focus. Instead, I engaged in close, detailed conversation with beneficiaries in an effort to acquire a more rich understanding of their experiences and interpretations of Medicaid policy. Those conversations yielded vital insights and are especially significant given the glaring absence of Medicaid beneficiaries’ voices from much of the research attempting to explain outcomes in their lives.⁴

The interview cases were purposively selected with the goal of achieving a range on the basis of race, gender, and geography (Yin 2003). Among the beneficiaries I interviewed, 82 percent were women, 42 percent were African-American, and 16 percent were Latino. They hailed from thirteen states and represented a variety of regions within those states (urban, rural, and suburban).⁵ The interviews were audio recorded, transcribed, and thematically coded.⁶ Analyses of the resulting transcripts uncovered the ideas that ground my understanding of the heterogeneity of Medicaid’s micro-political effects. In light of a dearth of knowledge about the experiences of Medicaid beneficiaries, and given research indicating that perceptions are an important mechanism through which geographic context affects political participation, it was fitting to center the views of beneficiaries as a starting point (Michener 2013). Still, I resist making strong causal claims based solely on the interviews. Instead, I leverage the qualitative data as one form of important evidence to gain clarity on the processes that shape Medicaid beneficiaries’ political lives and to develop focused quantitative models (Auerbach and Silverstein 2003; Small 2011).

The interview data corroborate and extend the political learning approach advanced by Joe Soss (1999, 2000). In explaining the relationship between welfare receipt and political involvement, Soss’ framework emphasizes the actual experience of receiving benefits. The core logic is that salient interactions with the government convey important political

lessons. Critically, policy experiences and their corresponding lessons differ by design. Soss (1999, 2000) first focuses on the broad distinction between means-tested and non-means-tested designs, arguing that the former teach welfare beneficiaries to view government in ways that undermine external political efficacy and thereby dampen participation. In later work, Bruch, Ferree, and Soss (2010) look beyond the means-testing dichotomy to differentiate between paternalistic (cash assistance), bureaucratic (public housing), and incorporating (Head Start) means-tested policy designs.

Building on these insights, I consider heterogeneity *within* a single means-tested program (Medicaid) based on variation across states. I argue that 1) the adequacy; and 2) the capriciousness of material benefits are crucial purveyors of differential policy-based learning. More specifically, beneficiaries consistently highlighted two imperative political lessons: that government lacked the resources to meet even basic needs (inadequacy), and that government could not reliably provide benefits (capriciousness). Having become sensitized to these shortcomings, Medicaid beneficiaries were generally unenthusiastic about the primary means of influencing government: politics. Most crucially, since the adequacy and capriciousness of Medicaid resources vary by state, these political learning mechanisms produce geographically differentiated effects.

A View from Policy Beneficiaries: Learning Government Inadequacy and Capriciousness

Nearly every interviewee I spoke to initially said positive things about Medicaid, echoing a recent Gallup poll showing that 76 percent of Medicaid beneficiaries are “satisfied” with how the health care system is working for them (Searing 2014). They described Medicaid as “awesome,” “great,” “a lifesaver,” and so on. Willy, a man in his late thirties who had had a heart attack when he was twenty-five and had been on Medicaid ever since, illustrates beneficiaries’ most general evaluations: “I have been poor my whole life, and without Medicaid I would be sick, homeless, and dead probably ... so I would just say that I think Medicaid is great.” Similarly, Christy, a fifty-year-old mother of eight, declared, “I am blessed to have Medicaid ... I am satisfied to the utmost.” Willy, Christy, and many others might have given Medicaid eight, nine, or even ten points on a ten-point scale. But at the slightest probing, more thorough descriptions of the program often pivoted to perceptions of inadequacy and capriciousness. Take Willy, for example; despite his overall

satisfaction, he struggled with getting transportation to and from the local Medicaid office and did not understand why Michigan could not cover transportation services. Likewise, though Christy stressed her blessings, she later explained that with Medicaid:

There are some things you can get [and] some things you can't, like glasses; I can't see. You know they say go to the dollar store and get some reading glasses ... but now my eyes are getting worse and worse. Medicaid told me that I can get a free eye examination, but not glasses.

Accounts like this were common. Efrain's experience underscores as much. In 2011, Efrain enrolled in Florida's Medicaid program. The coverage he was given proved a godsend. He was finally able to see doctors and was eventually diagnosed with diabetes. As a result, his "opinion of Medicaid [was] very positive." He explained that "the access in many cases is very good ... so I haven't had a problem ... everyone is very nice." Despite these positive assurances, before long Efrain mentioned his disappointment that in recent years Florida had cut back on dental coverage: "I miss the Medicaid when there is none, like in the case of the dental program, which is very reduced – they never give you anything, basically." Efrain also discussed program changes that happened in 2013.⁷ Around that time, making appointments with specialists (like his urologist) became much more difficult and filling prescriptions became a burdensome task. Efrain recalled that for the first time at that point, Medicaid would not cover his prescription costs. His pharmacist realized that a medication was being refused because the program had recently stopped covering treatments for ear problems. To circumvent this, the pharmacist gave Efrain the precise same prescription – but for his eyes. It turned out that the same drugs could be used to treat either eyes or ears. So the pharmacist put in a prescription for eye care, but instructed Efrain to use it for his ears. While Efrain appreciated the ingenious solution, he was struck by Florida's seemingly arbitrary coverage parameters.

This kind of epiphany was par for the course among Medicaid beneficiaries. Kay – a mother of four whose youngest son, Brian, was born prematurely and with severe disabilities – relayed an especially arresting story. At the time we spoke, Kay was the director of nursing at a health care facility and her husband was also a working professional. When Brian was born, Kay and her family had private health insurance and Brian was covered under it. But his health care expenses began skyrocketing immediately. Kay noted that his "NICU⁸ stay, just the bed alone, not counting any of the doctors' visits or surgeries or medications or

anything, just for the bed, was \$532,000.” When Brian’s numerous health conditions were diagnosed, his nurses immediately encouraged Kay to sign him up for Medicaid. Had Brian not qualified, Kay and her husband’s middle-class income would not have been sufficient to pay for the many medical needs not covered by their private insurer. Not only did Kay know this, she was acutely aware of the seemingly arbitrary terms of Brian’s eligibility:

We filled out all of that paperwork and then Brian was in the neonatal intensive care unit for four months. So what we then learned was there were different kinds of technicalities for a child to automatically qualify for Medicaid if they’re born prematurely, so born at thirty-one weeks or less and weighing two pounds, ten ounces or less automatically qualified. So Brian was born at thirty-one weeks and he weighed two pounds, ten ounces. *So by the grace of God*, he automatically qualified for that, which ended up being – he ends up costing about a million dollars a year in health care costs. Yeah, he averages about a million dollars a year (emphasis mine).

In Kay’s view, it was only *by the grace of God* that her family dodged the bullet of being saddled with impossible health care costs. When I asked her what would have happened had Brian been born at *thirty-two* weeks or weighed two pounds, *eleven* ounces, she noted that “there are a lot of parents who fall into that gap and end up paying – have to pay that off, because their private insurance doesn’t cover it.” Only “grace” had allowed her family to avoid this disastrous policy gap. Still, she was exceedingly grateful because Medicaid saved Brian’s life.

Ultimately, the simple story of high “consumer satisfaction” proved misleadingly superficial. Beneficiaries were appreciative for what Medicaid provided, and such gratitude was apparent in their general evaluations of the program. At the same time, they were painfully aware of what Medicaid *did not* provide and perceptively mindful of the arbitrary and shifting boundary lines between provision and non-provision.

Note that lessons about adequacy and capriciousness did not emerge from the sheer weight of poverty (i.e., I am poor, therefore I just do not think the government is doing enough for me), but were instead connected to specific (usually geographically rooted) policy experiences. Attesting to this, beneficiaries from a wide range of backgrounds expressed similar and consistent sentiments. Approximately 30 percent of the people I interviewed were, like Kay, from otherwise middle-class backgrounds. They relied on Medicaid for a variety of reasons (a chronic illness, a recent job loss), but they lived in middle-class neighborhoods, grew up with parents who were professionals (and/or were professionals themselves), and had

little experience with traditional welfare. Nonetheless, when these beneficiaries found themselves navigating Medicaid, they were still attuned to a lack of resources and dependability. Altogether, Medicaid beneficiaries across a range of race, class, and gender categories regularly expressed that government was inadequate and capricious (though to different effect, which I discuss in subsequent chapters).

Perhaps most vitally, beneficiaries shrewdly related these cognitions to politics. Angie, for example, spent much of our interview detailing all of the difficulties she had with Medicaid in Michigan. When I asked her if those experiences had anything to do with politics, she quickly declared, “Yes, it has a lot to do with politics ... I don’t think the average everyday working person really can influence politics much ... I don’t do politics.” Will was resigned to the fact that “if they are going to make cuts, there is not much that people [can] do about it but accept it.” Darius lamented his intermittent ineligibility for Medicaid and chalked it up to “politics,” which he then described as “like a double-edged sword. I mean it helps and then it is also dangerous too ... it requires a lot.” Likewise, Efrain thought that politics was “very important ... because it changes things,” but he suspected that Medicaid beneficiaries had “very little influence” in politics. When I asked Kay about how her experiences with Medicaid shaped her views of politics, she conveyed a deep sense of cynicism:

ME: Do you think your experiences with Medicaid have changed anything in terms of the way you view politics ... has it shaped your thoughts about politics, your feelings?

KAY: It has brought to light the dirty side of it and that it’s corrupt, and that this battle in Iowa, when it comes down to it, isn’t about what is best for Iowans; it’s a political game. That’s all it is. This is an election year, and the political game is disgusting, it’s filthy, it’s unethical. It’s *House of Cards* dirty. That’s what makes me sick, is that if people only knew that the intentions are not pure, what’s driving this is greed in politics. It’s dirty. And that is just disgusting.

Again and again beneficiaries proffered perspectives on the connections between politics and Medicaid, linking the program’s capriciousness and inadequacy to perceptions of political disempowerment. While a small handful reacted to such perceptions by delving more deeply into policy advocacy (like Kay, whose advocacy I discuss in Chapter 7), most responded with distrust and disengagement. Even the engaged beneficiaries viewed themselves as exceptions rather than the rule. Instructively, when Kay “changed hats” in our conversation, shifting from centering on her experiences as the mother of a beneficiary to her role as a nurse, she acknowledged that “if you put it on my nurse hat ... managing

people and caring for [beneficiaries] ... I don't see people advocating for themselves or even taking politics seriously."

MEDICAID AND POLITICAL PARTICIPATION: A QUALITATIVE VIEW OF STATE HETEROGENEITY

Even as beneficiaries stressed capriciousness and inadequacy, geographic positioning was central to their narratives. Remember Terrie from the first chapter? I began my interview of her with the open-ended question: "Tell me a little about yourself." In response, she noted "the difference between state to state with Medicaid and what it offers and the programs and how consistent they are." Like Terrie, nearly every beneficiary referred to Medicaid in state or local terms, and many enumerated specific aspects of state programs. Lucy, describing her former state of residence, insisted that "in Mississippi, they're still behind time ... it's who you know; it's not what you need." Lydia relayed that "in New York, assistance is easier to get than out here in Georgia." Riley contemplated moving away from Iowa to get better mental health services. Frank too confessed that only his penury kept him in Florida where Medicaid was paltry. Again, Kay was an uncommon but important example: her family actually did move from Iowa to Minnesota, leaving behind middle-class jobs and a home they owned in order to access the superior Medicaid benefits Minnesota promised to offer Brian.⁹ Kay more generally connected this move with another life-altering experience of Medicaid's geographic variation:

You know, after our son was born, we looked at moving to Tennessee because my parents live there and we would have had a home to live in that we wouldn't have had to pay for, just so that they were close by to help us. But they [Tennessee] don't have a waiver program, so the only way our son would have been able to get Medicaid was if we lived under the poverty line, so then we wouldn't work. Well then if I didn't work, I wouldn't be in homecare, and it was like, that makes no sense. And so I kept going around and around and basically learned there was no winning there. So we couldn't move there for our family to help us [but] we're actually going to be moving to Minnesota here in another month because ... their Medicaid is set up and designed and run better, so when this MCO thing came to light [in Iowa] ... it became a very easy decision to go. I don't have to fight anymore. Screw it, let's go. And so we're moving.

Kay was made painfully aware of which states had "waiver programs" that would allow disabled children in middle-class families to qualify for Medicaid. The lack of such a program in Tennessee prevented her from moving there to live near her parents during a time of dire need. But even

in Iowa, where Brian benefited from a waiver policy, there was a new “MCO thing.” MCO is an acronym for “Managed Care Organization.” Iowa had recently decided to transition from a state-run Medicaid program to a managed-care model that would put the administration of Medicaid in the hands of private entities contracted by the state. The privatization of Medicaid was a problem for Kay largely because of the unpredictability of the upcoming transition to managed care: it was unclear what changes there would be to coverage, whether Brian’s doctors would be included in coverage networks, and whether he would have to get special authorizations before he could receive care. Kay found this capriciousness unbearable and it was eventually the final straw that led her family to migrate to Minnesota.

While Kay’s case is unique, I found that even when beneficiaries had not moved much from state to state, they had friends, family, and social contacts who enabled them to compare experiences. Brianna, for example, lived in upstate New York for most of her life, but her two sisters and mother were also on Medicaid. Brianna’s mother recently migrated southward to Kentucky and managed to enroll in Medicaid there. She subsequently told Brianna all about how much less generous Medicaid benefits were in Kentucky. When Brianna and her mother contrasted the specifics of their state Medicaid programs, both women gained an entry point for understanding cross-state policy differences. Many of the other beneficiaries I spoke to described a similar dynamic. When asked how they thought Medicaid differed across states, nearly all of them offered details based either on their own experiences or on those of someone they were close to.

States are the teachers that draft the curriculum on which policy beneficiaries’ political learning is based. People learn experientially about the inadequacy and capriciousness of their states’ Medicaid policy. In particular, beneficiaries’ narratives frequently highlighted four issues: 1) benefit expansions; 2) benefit contractions; 3) the scope of services offered; 4) administrative capacity. Accordingly, these are the variables that I expect to be associated with political behavior among beneficiaries. In what follows, I draw on examples from interviews to delineate hypotheses related to each factor.

Benefit Contraction or Expansion

Changes in the benefits covered by Medicaid make a strong impression on people who rely on the program. Trajectory is especially important

because the status quo becomes more visible once it is altered. When people who were once unable to visit the doctor are now able to do so, they are more liable to develop positive views of government even if they previously felt negatively, and vice versa. Shana, a young woman from Michigan, ardently maintained that “I love Medicaid because ... it helps low-income families that can’t afford insurance.” She then followed up that sentiment with an observation that “the policy is changing ... making it hard for anyone to pretty much go to a doctor, just to get a general pap exam or any kind of exam.” Across several years, she could chart a process of retrenchment, recalling a downward trend that worried her. Most vitally, she directly implicated politics, declaring that “politics is what matters the most ... if you need services that you can’t afford, like Medicaid.” Likewise, John observed:

Over the past few years, it’s kind of becoming more of a struggle, you know. I think the state, they’re cutting back on a lot of benefits for people and they’re trying to make it harder and harder to where you just give up and you just feel like, well, I don’t want to do that or it’s not worth it to me, and they, you know, they figure if we can eliminate 10 people out of 100 that applied, you know, there’s savings for the state and all that. I understand that, that there is no money and there’s no, it’s not a lot of help ... again the past couple of years, I kind of don’t know what changed; it’s probably just like a budget issue, but I’ve run into a problem ... I’ve got a feeding tube and they don’t want to pay for the formula anymore, they don’t want to pay ... And the dietician is even fighting like tooth and nail and the doctor is saying, “he needs so many calories, you know, he’s underweight, he’s sick, he’s blah, blah, blah,” but they just have this category in their head.

John connected benefit retractions to the state he lived in (Michigan) and identified inadequate resources (i.e., the budget) as the culprit. Moreover, the loss of benefits was capricious insofar as it did not correspond to changing needs. Nothing about John’s health had changed over the past few years, but he nonetheless could no longer rely on the state to provide a vital need. Beneficiaries frequently exhibited this kind of knowledge of changes and assigned responsibility to the state for such shifts. When programs retract, they view state government as both capricious and inadequate. As such, I hypothesize (H₂) that beneficiaries living in states that have recently retracted Medicaid benefits will be less likely to participate in politics (relative to beneficiaries living in states that have not retracted benefits). Alternatively, when programs expand, beneficiaries view government through a lens of adequacy. Given this, I hypothesize (H₃) that beneficiaries living in states on an expansionary trajectory will be more

likely to participate in politics (relative to those living in states that have not expanded benefits).

Scope of Services Offered

As is typical in the U.S. federal system, there are many optional benefits that the national government does not mandate, but that states can nonetheless choose to provide. Some of these make a major difference in the lives of beneficiaries. Dani, a young mother from Michigan, said she was treated fairly and respectfully during the three years she was on Medicaid, but later told me, “I kind of wished they offered a few more services.” When I asked what kinds of services, she promptly answered:

Like vision. They don’t do any eye care ... unless you have low vision ... I think contacts are a luxury, so to speak, but I just feel like glasses are not. That’s definitely something that I think is needed to work. I can’t see. I couldn’t drive, no way. I can’t see; things are blurry to me twenty feet in front of me, ten feet in front of me. I’m not considered low vision because low vision is like almost next to blind, so I don’t get that.

Dani easily saw the political dimensions of this:

It’s the Democrats representing the people [who] need social services. You know what I mean, but they’re not Democrats [who] ever have even been in need of social services. I don’t know; for some reason, it’s like that population of people [who] are utilizing the services just – they don’t even have a voice.

Many beneficiaries mentioned dental and vision services. Terrie emblematically explained the benefits and disadvantages of her particular health plan, saying, “With Well Care I pay for 30 percent dental and vision is free. We have bad eyes, so I’ll take free vision and pay 30 percent dental ‘cause we have great teeth. You see what I’m saying?” Echoing this, Kyra told me, “I need braces because I grind my teeth, but they don’t pay for it. Both my kids need braces and they don’t pay for it ... Medicaid needs to ... you know, get people ... where their teeth are healthy.” Brianna and Maggie were especially bothered by Medicaid’s refusal to cover root canals (in New York and Michigan, respectively). This was memorable for both of them because it was a painful dental episode for which they had limited recourse to relief. Maggie recalled that “they don’t pay for root canals; you can only get them pulled, that’s it ... and eventually get dentures once you are approved.” Brianna believed that Medicaid did not pay for root canals because it was “cheaper to just take it out” rather than

try to “save the tooth.” Brianna had an entire row of teeth removed as a result of this. Maggie mentioned dental issues numerous times throughout our hour-long interview. She was openly perplexed over Medicaid’s refusal to acknowledge the importance of oral health.

The scope of services that beneficiaries were aware of did not stop at dental and vision. Maggie told of a skin condition: “I have vitiligo, which causes my skin to lose its color pigmentation. They consider that a cosmetic issue, so they won’t cover the cream I need for it.” Greg could not get inpatient expenses covered for his ankle surgery. Lucy even noticed limits on the amount of ice cream and juice that Georgia’s Medicaid would cover after her seven-year-old daughter got a tonsillectomy. It seemed as though there was no end to the nuances of optional services. Beneficiaries were cognizant of every nook and cranny and keen on the stakes for their families. Moreover, they identified states as the locus of Medicaid policies. When asked whether Medicaid was about national, local, or state politics, Maggie, Brianna, Lucy, and Greg each pointed to states. But in the face of miserly state resources, they often felt little power, echoing Maggie: “there is nothing you can do ... politicians have the control ... not beneficiaries.” Considering this state of affairs, I hypothesize that when non-mandatory benefits are narrow in scope, beneficiaries view the state through a lens of inadequacy and will be less likely to participate in politics (H4).

Administrative Capacity

Interviewees frequently relayed the extent to which Medicaid was overburdened. There were too many “clients” and too few “workers.” This state of affairs made the caseworkers hardened and unresponsive. To my surprise, beneficiaries did not usually blame street-level bureaucrats (even when they described negative interactions with them). Instead, they expressed a modicum of sympathy for bureaucrats. Maggie, for example, noted that “[caseworkers] have a hard job with a lot of clients ... I can only imagine how difficult their job must be trying to deal with so many individuals and remember so many different cases.” Similarly, when I asked Dani about her caseworker, she told me, “I mean you can tell that she is just busy and bogged down by caseloads up to the ceiling, you know what I mean, just like hundreds and hundreds of people that she is taking care of.” In beneficiaries’ view, caseworkers were trying to make lemonade with the sour lemons given to them by the “powers that be” (as Angie described it). Having little to work with, street-level bureaucrats

cannot help but succumb to the stresses of the job. As Lucy explained, the issue was fundamentally one of capacity: “It’s not enough caseworkers ... You have six, seven caseworkers to all these people, but instead of trying to break it down or hire more caseworkers, they get to you when they can get to you. You just better hope you have everything ready to go when they call you. If not, there’s another ten to twelve days.”

These kinds of observations convinced beneficiaries of the inadequacies of the government, over and above the failings of its individual employees. Illustratively, Shana, the Michigan woman I mentioned previously, praised the geniality of her interactions with Medicaid bureaucrats, saying, “I always have a good experience.” But as she later recounted the difficulty of reaching anyone from the Medicaid office on the phone, she drew larger conclusions about the limits of government, noting that with the government, “you can’t have your cake and eat it too” because “they can only do so much.” Human resource scarcity dampens beneficiaries’ views of government adequacy. As such, I expect that beneficiaries living in states with a low ratio of public welfare workers to people living in poverty will be less likely to participate in politics (H5).

QUANTITATIVE EVIDENCE OF THE MEDICAID-TO-PARTICIPATION LINK

To quantitatively test my five hypotheses, I use data from the third wave (2001/2004) of the Fragile Families and Child Well-Being Study (FFS).¹⁰ This survey follows a cohort of nearly 5,000 children born in U.S. cities, interviewing both mothers and fathers around the time of their child’s birth and at various intervals subsequently. Since the sample was composed to reflect nonmarital births in large U.S. cities, it is not nationally representative in the traditional sense. However, the emphasis on “fragile families” means that FFS contains an unusually large number of persons living in poverty and just under 4,000 Medicaid beneficiaries (53 percent of the sample).¹¹ In using this sample, I follow other scholars who have sought to understand the political behavior of disadvantaged Americans (Bruch et al. 2010; Weaver and Lerman 2010). Importantly, however, Medicaid beneficiaries in the FFS sample are not reflective of the (adult) Medicaid population nationally. FFS beneficiaries are younger, healthier, more likely to be male, and less likely to live below the poverty line. Since these are the characteristics of the beneficiaries who have recently joined Medicaid as a result of the expansion spurred on by the ACA, the patterns observed among FFS respondents are indicative of what we might

expect from expansion-based beneficiaries. To address concerns about this unique sample, I reproduced the initial findings using an alternative source of data (the National Longitudinal Study of Adolescent Health).¹² Doing so generated comparable results.

In the absence of an experimental intervention that randomly distributes Medicaid benefits to some eligible individuals while withholding them from others, it is not possible to make an unequivocal case for Medicaid's political effects.¹³ Still, working with available data, I take numerous steps to conduct robust analyses. After examining an initial set of regression models, I expand them to include pivotal but often overlooked factors that are principally relevant to low-income people. Next, I use a quasi-experimental *matching* technique to control for the confounding influence of factors causally unrelated to the primary independent variable. Finally, I use *seemingly unrelated regression* to mitigate concerns about who selects into the Medicaid program.

Measuring Political Participation

The models I present here estimate voter turnout, voter registration, and a more extensive index of political participation.¹⁴ Though I selected these dependent measures because of availability, it is also worth noting their substantive implications. Registering to vote and voting are central components of democratic citizenship, and they signal the most formal engagement with the political system. It is vital for scholars to understand the conditions that cultivate formal engagement among people on the economic and racial margins of American society (as many Medicaid beneficiaries are). The participation index additionally accounts for membership in a political group and taking part in a political rally. These are less conventional forms of involvement; both incorporate political action that extends beyond the bounds of electoral politics. Among groups who are politically disaffected, the inclusion of non-electoral participatory channels is imperative (Michener and Wong 2018). Of course, the measures of participation available via the FFS data are not exhaustive. In later chapters I examine other forms of political behavior including resistance to bureaucratic authority, local community activism, and policy advocacy.

Accounting for Other Factors

The main predictor of interest is a dichotomous measure of Medicaid enrollment.¹⁵ Additional controls reflect classic insights from the political

participation literature (Rosenstone and Hansen 1993; Verba, Schlozman, and Brady 1995) by focusing on key individual characteristics (age, education, employment, income, sex, race, nativity, cash welfare receipt, and civic attitudes¹⁶), as well as mobilizing factors (church attendance). I also include controls for personal health and child health.¹⁷

Some Initial Results

I begin with H1 and the general question of the Medicaid-to-politics link. This is not my main focus, but it is useful to evince that there is a correlation between Medicaid enrollment and political participation and, as per my qualitative indications, that it is negative. To do so, I predict each political outcome as a function of Medicaid enrollment plus the controls. The vote and registration models were estimated via probit and the participation model via ordinary least squares.¹⁸ As shown in Table 4.2, compared to the rest of the FFS sample, respondents who indicated being Medicaid beneficiaries are significantly less likely to vote, register, and participate more generally.¹⁹ A simple regression (column 1) confirms H1, indicating that Medicaid enrollment corresponds to a five-percentage-point decrease in the probability of voting, a five-percentage-point decrease in the probability of registering, and a six-percentage-point decrease in the probability of participation.²⁰

To bolster our confidence in these results, I conduct three robustness checks. First, following Bruch, Soss and Ferree (2010), I expand the models to incorporate factors often omitted from traditional studies of political behavior but known to have strong bearing on the lives of disadvantaged persons. These included drug and alcohol dependence, depression, incarceration, the number of kids in a household, and marital status. Though many of these prove significant (especially the measure for incarceration), adding them did not attenuate the significance of Medicaid (see Table 4.2, column 2).²¹

Next, I present the results after using coarsened exact matching (Iacus, King, and Porro 2012). Matching attempts to address the confounding influence of factors causally prior to the primary independent variable by better aligning the distributions of observed covariates in the treatment group (Medicaid beneficiaries) with those in the control group (non-beneficiaries). As shown in Table 4.2 (column 3), statistical models using matched data indicate that the earlier findings hold.²² This means that compared to non-beneficiaries who are similarly situated in terms of age, sex, receipt of cash assistance, education, race, marital status, and

TABLE 4.2 *Estimated Effects of Medicaid on Political Outcomes*

Outcome	Simple Regression (1)	Expanded Models (2)	Coarsened Exact Matching (3)	Seemingly Unrelated (4)
Vote	-0.10* (0.05)	-0.12* (0.05)	-0.07* (0.04)	-0.37* (0.14)
Register	-0.12* (0.04)	-0.17* (0.05)	-0.09* (0.04)	-0.34* (0.13)
Participation	-0.06* (0.02)	-0.07* (0.02)	-0.03* (0.01)	-0.39* (0.12)
N	5,033 (<i>Vote</i>) 7,079 (<i>Register</i>) 7,113 (<i>Participate</i>)	4,955 (<i>Vote</i>) 6,915 (<i>Register</i>) 6,948 (<i>Participate</i>)	3,925 (<i>Vote</i>) 5,543 (<i>Register</i>) 6,563 (<i>Participate</i>)	3,299 (<i>Vote</i>) 4,663 (<i>Register</i>) 4,426 (<i>Participate</i>)

Cells report coefficients from Medicaid variable, with standard errors in parentheses. Asterisks denote point estimates that were statistically significant at the $p < 0.10$ level (two-tailed tests). All models include full array of controls described in text

income, Medicaid beneficiaries are still significantly less likely to participate. There are a host of reasons why demographically similar survey respondents may not be enrolled in Medicaid: they may not have encountered an institution that encourages them to enroll (many beneficiaries join through hospitals or clinics), they may not know that they are eligible (many eligible people fail to enroll; see Sommers et al. 2012b), they may view Medicaid as stigmatizing and thus decline to enroll (Levinson and Rahardja 2004), or they may not be able to navigate sometimes complex enrollment procedures (Stuber and Kronebusch 2004). Instructively, most of the factors that explain non-enrollment among a group that is demographically similar to Medicaid beneficiaries point to such a group being in a *weaker* position vis-à-vis political engagement (e.g., out of contact with community institutions and social networks, vulnerable to feeling stigmatized, having limited knowledge of bureaucratic processes). In other words, unless Medicaid itself is affecting political behavior, we should expect a group of people who are demographically similar to Medicaid beneficiaries but not enrolled in the program to be *less* politically active. That means that the bar for finding a negative relationship (in the empirical context of a matched sample) was high, which makes the results even more compelling.

Finally, in the vein of considering barriers to enrollment, I offer a set of Seemingly Unrelated Regression (SUR) models aimed at addressing the possibility that unobserved factors influence *both* the decision to get Medicaid *and* the decision to vote. If it turns out that, contrary to

what I suggested earlier, people who decide to enroll in Medicaid somehow share an unmeasured trait that makes them less likely to vote, then the SUR models should capture this. As shown in Table 4.2 (column 4), estimating the models via SUR does not change the findings: Medicaid remains a significant and strong predictor of political behavior.

MEDICAID AND STATE POLICY CONTEXTS

The empirical support I provide for H_1 is not incontrovertible, but it is strong evidence that Medicaid enrollment has an overall negative individual-level correlation with political participation. Recall, however, that my qualitative work led me to question the geographic patterns underlying this overarching relationship. Closer scrutiny confirms the validity of this line of inquiry. For example, separate (matched) regression models for each of the fifteen core states represented in the Fragile Families data indicate that the relationship between Medicaid and political participation exhibits wide geographic variation (see Figure 4.1). There is a negative and significant association in New York, California, and Virginia, a positive relationship in Wisconsin, and no significant association in Texas, Michigan, New Jersey, and others. Note that Medicaid can be a boon to participation. Though the examples offered earlier stress beneficiaries' adverse experiences with the program, it is worth noting that such outcomes are not inevitable. If we are to grasp how to avoid the negative gradient, it is crucial to identify the policy factors that contribute to it. Guided by the themes identified through the in-depth interviews, I turn to the quantitative data to do so.

The Political Effects of State Policy Heterogeneity

Recall that the political learning approach outlined earlier focuses on the lessons that beneficiaries are taught about the adequacy and capriciousness of state Medicaid policies. In particular, I hypothesized that four factors would prove especially educational for beneficiaries: 1) benefit expansions; 2) benefit retractions; 3) the scope of services offered; 4) administrative capacity. Benefit reductions implicate the capriciousness of the government, while benefit expansions, a limited scope of services, and human resource scarcity each implicate the adequacy of government (albeit in distinct ways).

In the models that follow, I explore the associations between these factors and the political participation of Medicaid beneficiaries. Note my

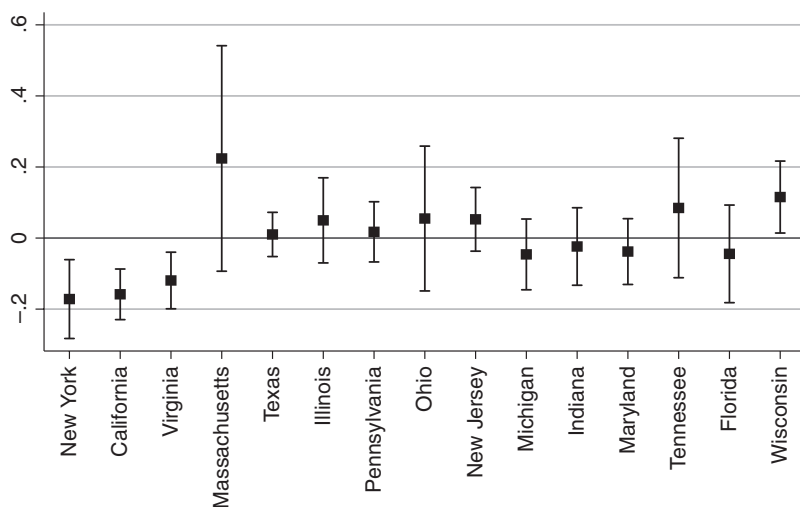


FIGURE 4.1 Medicaid and political participation across states

Note: Dots represent point estimates of the “effect” of Medicaid for within-state OLS models of political participation. Bars represent 90 percent confidence intervals. Models are based on matched samples and include controls.

transition from employing models that compare beneficiaries and (similarly situated) non-beneficiaries, to examining models limited to only beneficiaries. The political learning explanation posits that the political influence of state policies depends upon the experience of Medicaid policy and the lessons it confers. As such, I expect that only Medicaid beneficiaries’ political participation will vary based on the specific kinds of policy heterogeneity emphasized here. Since some of these factors may also affect participation among disadvantaged *non-beneficiaries*, I keep a comparative eye on equivalent models among that group as well. This is a placebo test. To the extent that the expectations outlined earlier are borne out *more so* among beneficiaries than among their matched counterparts, it is evidence of the ways that states help turn policy into politics by distinctively shaping the experiences (and therefore interpretive lenses) of policy beneficiaries.

In the quantitative models of state policy heterogeneity, the outcome variables and basic controls are the same as those presented earlier. I also control for the proportion of Medicaid enrollees who were black and the degree of within-state geographic health disparities.²³ The predictors of interest are measures that reflect the following state-level features: 1)

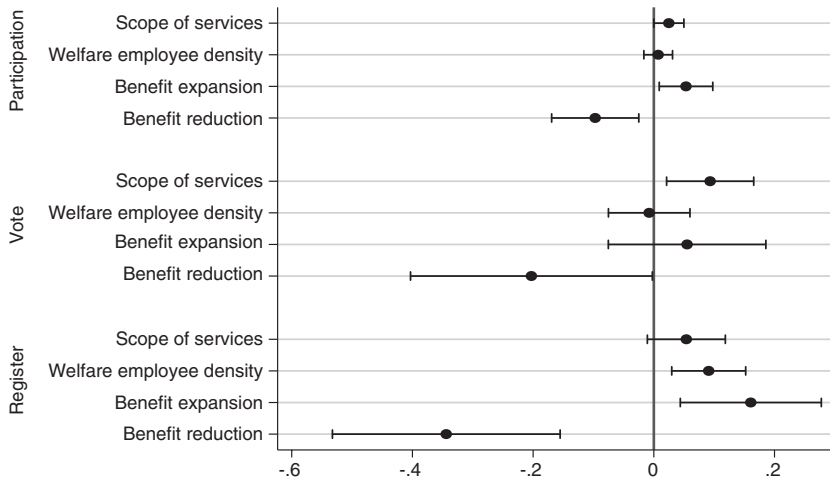


FIGURE 4.2 State policy factors and beneficiary political behavior

Note: Dots represent regression coefficients and bars represent 90 percent confidence intervals. Models are based on matched samples and include full array of controls. See Appendix for full tables.

whether a state had expanded the Medicaid benefits offered in the past year²⁴; 2) whether a state has retracted any benefits²⁵; 3) how a state Medicaid program ranks in terms of provision of optional health services²⁶; 4) the density of state welfare bureaucrats.²⁷ The models are limited to the fifteen core states contained in the Fragile Families data.²⁸ Figure 4.2 illustrates the main findings.

As shown, state policy arrangements can boost or dampen political action among Medicaid beneficiaries. Beneficiaries living in states that expanded benefits in the previous year are significantly more likely to register and participate more generally; those living in states with a higher density of welfare employees are substantially more likely to register; those in states offering a wider scope of optional services are more likely to vote; and those living in states that have recently reduced benefits are significantly less likely to participate, vote, and register.

The effect of benefit reductions was the largest and most consistent. To provide a sense of magnitude, Figure 4.3 displays three graphs showing the marginal effects of Medicaid policy reductions on each political outcome. As shown, compared to beneficiaries living in states that did not reduce benefits, beneficiaries living in states that had made

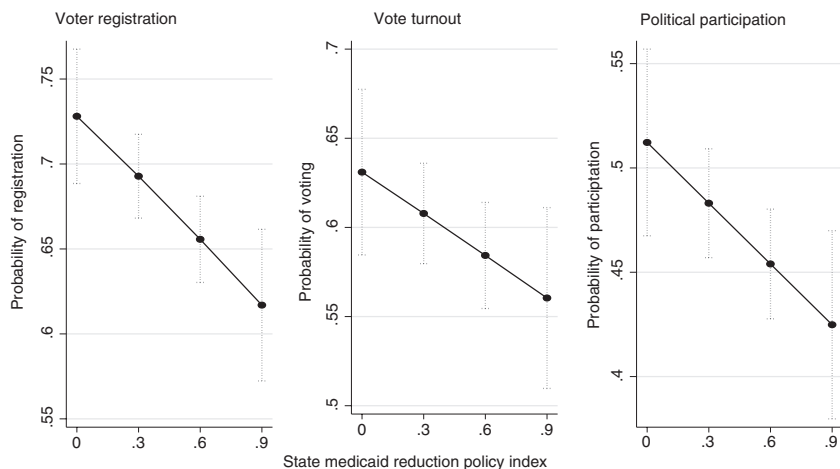


FIGURE 4.3 State Medicaid reduction policies and beneficiary political outcomes

the most reductions were between four and nine percentage points less likely to vote, register, or participate.

These results are substantively strong and consistent with expectations. They are also robust to many additional controls, including states' ideological leanings, a measure of state welfare paternalism, the size of the Medicaid population, the partisan composition of state legislatures, the proportion of noncitizens in a state, the degree of within-state income inequality, contextual demographics at the census tract level, and even the organizational structure of state Medicaid programs (i.e., whether they are linked to or separate from state public health, mental health, and human services departments). Crucially, most of the state policy variables highlighted in the models are insignificant when the models are run for similarly situated (i.e., matched) non-beneficiaries.²⁹ This suggests that the political behavior of *beneficiaries* is especially responsive to state policy factors that shape their experience of the program.

Of course, these findings are not dispositive. The Fragile Families Survey provides access to a uniquely large but nonetheless idiosyncratic group of Medicaid beneficiaries. Though similar analyses conducted using a very different sample showed equivalent results (not shown), caution is still warranted when generalizing beyond the sampled populations. In addition, while the empirical models cover much ground, they do not eliminate the possibility of inferential biases.³⁰

Finally, note that the quantitative models do not *explain* the general Medicaid-to-politics link (though they establish it as best as possible). While I aver, based on qualitative observations, that stigma and negative bureaucratic interactions are part of the reason for this link, most of my emphasis was on grasping the contours of state variation. Moreover, adding the state-level factors based on H2–H5 to the original (full sample) models (which estimated the general relationship between Medicaid and participation) does not render the indicator for Medicaid enrollment insignificant. If it did, then I would have evidence that state factors explain the general Medicaid-to-politics link. But that is not the case. Instead, accounting for state policy factors primarily helps us to understand the distributional patterning of Medicaid's relationship to political behavior.

The multi-method evidence presented in this chapter indicates that Medicaid's democratic reverberations are tethered to the federal political system. Federalism gives states the discretion to design policy (to expand benefits or to reduce them, to offer a broad or narrow scope of services) and to implement it (hiring enough workers to meet demand or failing to do so). This produces heterogeneity in Medicaid beneficiaries' experiences of policy, teaching pointed and differential lessons about the inadequacy and capriciousness of government in the face of need. Qualitative evidence indicates that this education (or miseducation) has political upshots, shaping the way that Medicaid beneficiaries view government and politics. Quantitative evidence supports this, revealing that the Medicaid-to-politics relationship varies across states based on policy configurations.

Political scientists have long argued that “policy makes politics.” I demonstrate some of the conditions under which *state* policies make particular kinds of politics. But federalism is not “just a system that allows states to pursue different strategies; it is also a relationship between levels of government” (Soss et al. 2011: 93). With this broader view in my sights, I turn to the counties that operate within states to consider their implications for Medicaid beneficiaries' political lives.